



UMEDA CO., LTD.

26/18-21 Ram-Indra Road, Bangkok 10230 Thailand

Tel. (662) 943-6366 Fax (662) 943-6989 E-mail address : umeda79 @ samarts.com

ISO 9001 : 2008

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PACKAGE INSERT

Brand or Product Name

PARACAP PARACETAMOL 120 MG/5 ML ORAL SUSPENSION

Name and Strength of Active Substance(s)

Paracetamol 120 mg/ 5 mL

Product Description

A Pink coloured with strawberry-flavoured in form of a suspension.

Pharmacodynamics

Paracetamol is a centrally acting analgesic and antipyretic with minimal anti-inflammatory properties.

Analgesic

The mechanism of analgesic action has not been fully determined. Paracetamol may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (specifically cyclooxygenase (COX)-2) and, to a lesser extent, through a peripheral action by blocking pain-impulse generation.

The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitize pain receptors to mechanical or chemical stimulation

Antipyretic

Paracetamol act centrally on the hypothalamic heat-regulating center to produce peripheral vasodilatation resulting in increase blood flow through the skin, sweating and heat loss. Paracetamol reduces fever by inhibiting the formulation and release of prostaglandins in the CNS and by inhibiting endogenous pyrogens at the hypothalamic thermoregulator center

Pharmacokinetics

Following oral administration paracetamol is rapidly absorbed.

Paracetamol absorption takes place mainly in the small intestine and therefore the rate of absorption is depending on the rate of gastric emptying. It has been shown that drugs which



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delay gastric emptying also delay the absorption of paracetamol whereas metoclopramide (a drug which increases the rate of gastric emptying) accelerates absorption of the analgesic through the total amount absorbed doses not increase.

The presence of food in the stomach has also been reported to reduce the rate of absorption of paracetamol. Alterations in gastric pH have no appreciable effect on paracetamol absorption. During absorption, the amount of paracetamol which is inactivated is negligible and it has been shown that paracetamol dose not affect gastric mucosal permeability and does not produce mucosal bleeding.

Peak plasma concentrations are reached 1 hour after absorption. The plasma half life is 1 to 3 hours.

Paracetamol penetrates the brain and is present in breast milk of human.

Paracetamol is metabolized by the microsomal enzyme system of the liver. This metabolism is mainly to the glucuronide and sulphate conjugates, accounting for approximately 49% and 26% of the ingested dose respectively. About 4% is excreted as free paracetamol. Other minor pathways include the production of catechol derivatives and cysteine conjugates (via glutathione). Paracetamol excretion is rapid and occurs via the urine.

Indication

For relief of fever and pain.

Recommended Dosage

For children aged 1 to 6 years. Give in 6 hourly intervals; not more than 4 times a day.

Age	Paracetamol Suspension 120 mg/5 ml
1 – 3 years	6 ml – 10 ml
4 – 6 years	10 ml – 15 ml

The above table is based on the average weight-age grid of children. If you know the body weight of your child, it's always more accurate to use the weight to arrive at the correct dose. Please refer below.

Using the body weight of your child, you can calculate the dose of Paracetamol Suspension 120 mg/5 ml your child needs. Calculation is based on recommended dose of 15 mg per body weight in kg (BW).

Table below provides the dose of Paracetamol Suspension 120 mg/ 5ml to be given, based on body weight:



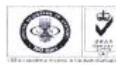
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Child's Weight		Paracetamol Suspension 120 mg/5 ml
In kg	In lb	
10	22	6 ml
15	33	9 ml
20	44	13 ml
30	66	19 ml

Note: 1kg = 2.2 lb

Mode of Administration

Oral

Contraindications

Hypersensitivity to paracetamol or any of the other ingredients/components of the product
Severe and active hepatic impairment

Warnings and Precautions

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.
Allergy Alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

- Keep out of reach of children.
- Do not take if allergic to paracetamol.
- Patients should contact their health care provider if symptoms persist (if the pain lasts for more than 10 days, if there is redness or fever lasts more than 3 days).
- Paracetamol should be given with care to patients with impaired kidney or liver function.
- Large doses should be avoided in patients with hepatic impairment. Paracetamol overdose may harm the liver.





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- Do not exceed recommended dose.
- Paracetamol provides symptomatic relief only, additional therapy to treat the cause of the pain or fever should be instituted when necessary.
- Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious condition. If these reactions occur, stop use and seek medical assistance right away.

Effects on Ability to Drive and Use Machines

It is unlikely to impair a patient's ability to drive or use machinery.

Interactions with Other Medicaments

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

Adverse Effects / Undesirable Effects

Adverse effects of paracetamol are rare and usually mild, although haematological reactions have been reported. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic epidermal Necrolysis have been reported.

Overdose and Treatment

Symptoms:

Toxic symptoms include vomiting, abdominal pain, hypotension and sweating. The most serious adverse effect of acute overdose of paracetamol is a dose-dependent, potentially fatal hepatic necrosis. Clinical and laboratory evidence of hepatotoxicity may be delayed for up to one week. Major manifestations of liver failure such as jaundice, hypoglycemia and metabolic acidosis may take at least 3 days to develop.



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**Treatment:**

In cases of overdose, methods of reducing the absorption of ingested drug are important. Gastric lavage is essential even if several hours have elapsed. Prompt administration of 50 g activated charcoal and 500 ml iced mannitol 20% by mouth, may reduce absorption. If the history suggests that 15 g Paracetamol or more has been ingested, administer one of the following antidotes:

Acetylcysteine 20% i.v.: Administer intravenously, 20% acetylcysteine (Parvolex, Glaxo) immediately without waiting for positive urine test or plasma level results: initial dose of 150 mg/kg over 15 minutes, followed by continuous infusion of 50 mg/kg in 500 ml 5% glucose/dextrose over 4 hours and 100mg/kg in 1L 5% glucose/dextrose over 16 hours; or

OR

Oral Methionine: 2.5 g immediately followed by three further doses of 2.5 g at four hourly intervals. For a 3 year old child, 1g methionine every four hours for four doses has been used;

OR

Oral Acetylcysteine 5%: 140 mg/kg as a loading dose, then 70 mg/kg every 4 hours for a total of 17 maintenance doses. If more than ten hours have elapsed since the overdose was taken, the antidote may be ineffective

Storage Conditions

Store below 30°C

Dosage Forms and Packaging Available

Oral suspension in a 60 mL plastic bottle (HDPE) into a paper box of 1 bottle

Name and Address of Manufacturer

UMEDA CO., LTD.

26/18-21 Soi Yuyen, Ram-Indra Road,
Moo 9, Taraeng, Bang Khen, Bangkok
10230, Thailand

Tel. (662) 943-6366 Fax (662) 943-6989

Name and Address of Marketing Authorization Holder

HEALTH PHARM MARKETING SDN BHD

30-4, Jalan 3/146, Bandar Tasik Selatan, 57000
Kuala Lumpur, Malaysia**Date of Revision of Package Insert**

30/10/2015

